

Most prolapse repairs done through the vagina use **your own tissue**, with stitches and no added material. This is called a **native-tissue repair**. Many people have heard worrying news about vaginal mesh. This sheet explains your choices and the history of mesh, so you can decide with your surgeon.

About This Procedure

The repair is done **through the vagina**, with no cut in the belly. The surgeon opens the vaginal wall and uses stitches to tighten and strengthen the weak support tissue. Which repair you need depends on the part that drops:

- **Front wall** (bladder): anterior repair, or anterior colporrhaphy
- **Back wall** (rectum): posterior repair, or posterior colporrhaphy
- **Top** of the vagina: a suspension to strong ligaments (see its own handout)

If you have a back-wall repair: trouble emptying the bowel, such as straining or needing to press to pass stool, often improves, but not always. In one long-term study, about 1 in 3 people still had some of these symptoms. Pain with sex can also happen afterward. Tell your team if it does.

Grafts and Mesh

Extra material can be added to these repairs. A **biological graft** is made from donated human or animal tissue. **Synthetic mesh** is a permanent woven material. In a large trial of over 1,300 women, adding a graft or mesh to a front- or back-wall repair **did not improve symptoms**. About 12 in 100 women who got mesh had a mesh problem within 2 years. Grafts are not routinely recommended for these repairs.

The History of Vaginal Mesh

The FDA warned about problems with mesh placed through the vagina for prolapse in 2008 and 2011. In **2019 it ordered these products off the US market**. The order does not cover mesh placed through the belly (sacrocolpopexy) or slings for urine leakage. If you already have vaginal mesh and no problems, it does not need to come out.

Learn the Terms

Prolapse

When pelvic organs drop and cause a bulge or pressure.

Native-tissue repair

A repair that uses only your own tissue and stitches.

Anterior repair

Repair of the front vaginal wall, under the bladder.

Posterior repair

Repair of the back vaginal wall, over the rectum.

Biological graft

Support material made from donated human or animal tissue.

Transvaginal mesh

Permanent mesh placed through the vagina. It is no longer sold in the US for prolapse.

Mesh exposure

When mesh shows through the vaginal wall. It can cause discharge, bleeding or pain with sex.

Sacrocolpopexy

A different repair that places mesh through the belly.

Should I worry about mesh? It is a fair question. A repair with your own tissue uses no mesh. The problems that led to the FDA order were with **mesh placed through the vagina for prolapse**. Ask your surgeon what material, if any, is planned and why.

First day

Walk soon after surgery. Your bladder emptying is checked.

First weeks

Spotting and discharge are normal. Keep stools soft. Return to activity at your own pace.

Until cleared

Nothing in the vagina and no sex until your team says so, often about 6 weeks.

Follow-up visit

Your team checks healing and how the repair holds.

Questions to Ask Your Surgeon

- Which parts will you repair, and will you use **only my own tissue**?
- What are my other choices, such as a pessary or a repair through the belly?
- How likely is the prolapse to **come back**?
- Will anything be done for urine leakage?

What Happens During Surgery

- 1 You get general or spinal anesthesia, and antibiotics.
- 2 The surgeon opens the vaginal wall, tightens the support tissue with stitches, and closes the wall.
- 3 A catheter, and sometimes gauze in the vagina, may stay in for a short time.

Will It Last?

Prolapse can come back after any repair. A small return seen on an exam is common, and many people do not feel it. Fewer need another operation.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Discharge that smells bad, or pain that is getting worse
- Later on, new bleeding, discharge or pain with sex

Go to an emergency department if:

- You **cannot urinate** for several hours and your bladder feels full or painful
- Bleeding that soaks a pad in an hour
- You cannot reach the office and are worried

Three Things to Remember

1. Most vaginal prolapse repairs use your own tissue, with no mesh.
2. Mesh placed through the vagina for prolapse has not been sold in the US since 2019. Belly mesh and leakage slings are different.
3. Ask exactly what your surgeon plans to use, and why.